

1. Administrative & Study Identification

**Full Study Title:** A Clinical Study of Belzutifan and Zanzalintinib in People With Recurrent Kidney Cancer Following Adjuvant Therapy (MK-6482-033) **Short Title/Acronym:** MK-6482-033 / LITESPARK-033 **Protocol Number:** MK-6482-033 (EU CT: 2024-517136-21) **NCT Number:** NCT07227402 **Posting ID:** 2307221516751451 **Trial Status:** Not yet recruiting (NOT\_YET\_RECRUITING) **Sponsor/Company Name:** Merck Sharp & Dohme LLC / Merck **Investigational Product:** Belzutifan (MK-6482) and Zanzalintinib (Active comparator: Cabozantinib [ATC Code: L01EX07; Trade Names: Cabometyx, Cometriq]) **Phase of Development:** Phase 3 **Medical Monitor:** [Information not explicitly available - Managed by Merck Clinical Operations]

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2. Study Synopsis & Rationale

**2.1 Study Objective Primary Objective:** To determine if recurrent advanced renal cell carcinoma (RCC) responds (gets smaller or goes away) and if patients live longer overall and without the cancer worsening after treatment with belzutifan and zanzalintinib compared to cabozantinib. **Secondary Objectives:** To evaluate the Objective Response Rate (ORR), Duration of Response (DOR), and overall safety and tolerability (Number of participants who experience one or more Adverse Events) of the combination therapy.

**2.2 Background & Rationale** Researchers are looking for more ways to treat advanced renal cell carcinoma (RCC) that is recurrent. This study aims to address the therapeutic gap for patients who experience disease recurrence during or following adjuvant anti-PD-1/L1 therapy, evaluating if patients on the belzutifan and zanzalintinib combination live longer overall and without the cancer getting worse than those taking cabozantinib.

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3. Study Design & Methodology

**3.1 Design Framework Study Type:** Interventional **Control Type:** Active Comparator (Cabozantinib) **Blinding:** Open-label / Investigator-blinded (Protocol specified) **Randomization:** Randomized (Belzutifan + Zanzalintinib vs. Cabozantinib)

**3.2 Planned Enrollment & Duration Target Enrollment (\$N\$):** 904 participants **Number of Sites:** Global Multicenter Trial **Estimated Study Start:** November 26, 2025 **Estimated Primary Completion:**

## 4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Age  $\geq 18$  years; All Genders. 2. Histologically confirmed diagnosis of unresectable, advanced renal cell cancer (RCC) with clear cell component (with or without sarcomatoid features), i.e., Stage IV renal cell cancer per AJCC (8th Edition). 3. Measurable disease per RECIST 1.1. 4. Disease recurrence during adjuvant anti-PD-1/L1 therapy or  $\leq 24$  months following the last dose of adjuvant anti-PD-1/L1 therapy. 5. Received no other prior systemic therapy for their RCC except for their adjuvant anti-PD-1/L1 therapy.

**4.2 Exclusion Criteria** 1. Clinically significant cardiovascular disease within 12 months from first dose of study intervention (e.g., NYHA Class III or IV congestive heart failure, unstable angina, new-onset angina, pulmonary embolism, myocardial infarction, cerebral vascular accident, or cardiac arrhythmia associated with hemodynamic instability). 2. Deep vein thrombosis within 3 months before randomization unless stable, asymptomatic, and treated with therapeutic anticoagulation for at least 4 weeks before randomization. 3. Left ventricular ejection fraction  $\leq 50\%$  or below the institutional (or local laboratory) normal range. 4. Major surgery within 8 weeks before randomization, or not adequately recovered from major surgery, or has ongoing surgical complications. 5. Current pneumonitis/interstitial lung disease. 6. Symptomatic pleural effusion (e.g., cough, dyspnea, pleuritic chest pain), ascites, or pericardial fluid requiring drainage within 4 weeks prior to randomization. 7. A gastrointestinal disorder including those associated with a high risk of perforation or fistula formation. 8. A serious active nonhealing wound/ulcer/bone fracture. 9. Requirement for hemodialysis or peritoneal dialysis. 10. History of human immunodeficiency virus (HIV) infection, hepatitis B, or hepatitis C virus. 11. Received a live or live-attenuated vaccine within 30 days before the first dose of study intervention.

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## 5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** Belzutifan + Zanzalintinib (Experimental Arm) OR Cabozantinib (Active Comparator Arm). Detailed dosing schedules per protocol. **Comparator Drug Details:** Cabozantinib (ATC Code: L01EX07, Trade Names: Cabometyx, Cometriq). Relevant EMA Product Information: [Cabometyx](#), [Cometriq](#). **Route of Administration:** Oral targeted therapies. **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, or study withdrawal.

**5.2 Concomitant & Prohibited Therapy Permitted:** Standard supportive care medications per investigator discretion.  
**Prohibited:** Live or live-attenuated vaccines within 30 days before the first dose; concurrent investigational anticancer agents.

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**6. Study Timeline & Visit Schedule**

| Visit ID     | Window (Days)           | Key Activities/Procedures                                          |
|--------------|-------------------------|--------------------------------------------------------------------|
| Screening    | Day -28 to 0            | Informed Consent, Medical History, Tumor Imaging, Baseline Labs    |
| Baseline     | Day 0                   | Randomization, First Dose                                          |
| Interim      | Protocol-defined cycles | Safety Labs, Efficacy/Tumor Assessment (RECIST 1.1), AE Evaluation |
| End of Study | Follow-up               | Final Safety Assessment, Survival Follow-up                        |

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**7. Outcome Measures (Endpoints)**

**7.1 Primary Endpoint Progression Free Survival (PFS):** Defined as the time from randomization to the first documented progressive disease (PD) per RECIST 1.1 (assessed by blinded independent central review [BICR]) or death due to any cause. PD is defined as  $\geq 20\%$  relative increase and  $\geq 5$  mm absolute increase in the sum of diameters of target lesions, or the appearance of new lesions. **Overall Survival (OS):** Defined as the time from randomization to death due to any cause.

**7.2 Secondary & Exploratory Endpoints** \* Objective Response Rate (ORR). \* Duration of Response (DOR). \* Number of participants who experience one or more adverse events (AEs).

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**8. Safety & Adverse Event Reporting**

**Adverse Event (AE) Monitoring:** Routine collection via patient interview, clinical examination, and laboratory assessments during study visits. Severity graded by standard oncology toxicity criteria (CTCAE). Number of participants experiencing  $\geq 1$  AE is a predefined secondary outcome. **Serious Adverse Events (SAEs):** Reported to the sponsor and regulatory authorities within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (IDMC) will oversee safety and efficacy data.

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**9. Statistical Analysis Plan (SAP) Summary**

**Analysis Populations:** Intent-to-Treat (ITT) for primary efficacy analysis; Safety Set for patients receiving at least one dose of the study drug. **Sample Size Justification:**  $N = 904$  is

statistically powered to detect meaningful differences in OS and PFS between the intervention and comparator arms. **Handling of Missing Data:** Handled via standard survival analysis methodologies (e.g., right-censoring for PFS and OS endpoints).

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## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

**Monitoring Plan:** Scheduled onsite and remote monitoring visits to ensure protocol adherence and data integrity. **Audit Trail:** Robust electronic Case Report Form (eCRF) systems with full data clarification forms and audit trails.

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## 11. Study Identifiers & Governance

**Primary ID:** MK-6482-033 **Registry Number:** NCT07227402 **Posting ID:** 2307221516751451 **Sponsor/Lead Organization:** Merck **Study Title:** A Clinical Study of Belzutifan and Zanzalintinib in People With Recurrent Kidney Cancer Following Adjuvant Therapy (MK-6482-033) **Official Regulatory Title:** A Phase 3, Randomized, Open-label Study of Belzutifan + Zanzalintinib Versus Cabozantinib in Participants With Advanced RCC Who Experienced Disease Recurrence During or After Prior Adjuvant Anti-PD-1/L1 Therapy (LITESPARK-033)

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## 12. Clinical Framework (Oncology/RCC Specific)

**Primary Condition:** Recurrent Renal Cell Carcinoma **Preferred UMLS Name:** Recurrent Renal Cell Carcinoma **Diagnosis Threshold:** Stage IV clear cell RCC with measurable disease (RECIST 1.1) **Medical Methodology:** Targeted Combination Therapy vs. Tyrosine Kinase Inhibitor (Cabozantinib) **Optimization Target:** Prolonged Overall Survival (OS) and Progression-Free Survival (PFS)

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## 13. Study Procedures & Methodology

**Management Pathway:** Standard oncological clinical trial pathway for recurrent advanced disease. **Education Protocol (HCP):** Site initiation visits; training on RECIST 1.1 guidelines and safety reporting. **Education Protocol (Patient):** Informed consent process, dosing instructions, and AE self-reporting guidelines. **Quality Audit Mechanism:** Blinded independent central review (BICR) for tumor imaging and standard sponsor QA audits.

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## 14. Observation & Follow-Up Schedule

**Total Duration:** Until study completion (estimated February 2032)  
**Onsite Visit Cadence:** Aligned with treatment cycles (e.g., Day 1 of every 21- or 28-day cycle) **Remote Monitoring Frequency:** Continuous survival follow-up post-treatment discontinuation  
**Checklist Review Interval:** Prior to administration of each new cycle dose

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## 15. Sign-off & Approval

| Role                     | Name   | Signature | Date          |  | :--- | :--- | :--- | :--- |
|--------------------------|--------|-----------|---------------|--|------|------|------|------|
| Principal Investigator   | [Name] | _____     | [DD-MMM-YYYY] |  |      |      |      |      |
| Clinical Operations Lead | [Name] | _____     | [DD-MMM-YYYY] |  |      |      |      | Lead |
| Statistician             | [Name] | _____     | [DD-MMM-YYYY] |  |      |      |      |      |